PERINATAL DEPRESSION AMONG VIRGINIA WOMEN

Trends and Public Health Implications 2016 to 2022

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Writing Sample

Note: This writing sample is an excerpt adapted from a longer academic paper.

Abstract

I examined changes in three indicators of depression among Virginia women from 2016 through 2022 using data from the Pregnancy Risk Assessment Monitoring System. The indicators were depression before pregnancy, depression during pregnancy, and postpartum depressive symptoms. Depression during pregnancy increased by 79.8%, depression before pregnancy increased by 46.2%, and postpartum depressive symptoms increased by 8.5%. These results show that maternal mental-health needs are not limited to the period after childbirth. Screening and support should begin before pregnancy and continue through pregnancy and the postpartum period.

Introduction

Perinatal depression can affect a woman's daily functioning, participation in prenatal care, relationship with her infant, and ability to care for herself and her baby.[1,2] Symptoms may be present before pregnancy, develop during pregnancy, or appear later in the postpartum period.[3] For this reason, focusing only on postpartum depression may leave some women without help. Current guidance recommends screening during prenatal and postpartum care, but screening and referral are not always carried out consistently.[4,5] Looking at Virginia trends can help show when support is needed and where services may need to improve.

Methods

I reviewed publicly available Virginia PRAMS estimates for 2016-2022.[6] For each indicator, I recorded the annual weighted prevalence and calculated the percentage change between the first and last years. I also compared the Virginia estimates with aggregate PRAMS estimates from participating sites. This was a descriptive analysis, so I did not use individual-level data or adjust for possible confounding factors.

Results

All three indicators increased during the study period. Depression during pregnancy had the largest change, rising from 9.4% in 2016 to 16.9% in 2022, an increase of 79.8%. Depression before pregnancy increased from 10.6% to 15.5%, while postpartum depressive symptoms increased from 12.9% to 14.0%. Across participating PRAMS sites, depression during pregnancy increased from 11.6% to 16.8%. Virginia was higher than the aggregate estimate in 2019, but the two estimates were nearly the same by 2022. These comparisons describe the trends but do not explain what caused them.

Discussion

The findings suggest that maternal mental-health care should not begin only after delivery. Waiting until a postpartum visit may miss women who already had symptoms before pregnancy or developed them during pregnancy. Prenatal care provides an important opportunity for screening, but screening by itself is not enough. Women also need clear referrals, timely assessment, appropriate care, and follow-up. Time limitations, inadequate training, stigma, and a shortage of referral options can make this difficult.[7,8] Telephone- and text-based approaches may help improve identification and treatment attendance.[9]

The increase before pregnancy also points to the importance of primary care, family planning, and preconception care. Better coordination between these services, obstetric practices, behavioral-health providers, and community organizations could reduce gaps in care. Access is also not equal. Cost, transportation, provider shortages, language barriers, and a lack of culturally responsive services can prevent women from receiving help. Previous studies have found differences in symptoms, diagnosis, and treatment across racial and ethnic groups and have linked neighborhood disadvantage with postpartum depression.[10–12]

Public Health Implications

Virginia public-health agencies can use PRAMS data to continue monitoring these trends and to identify groups that may need additional support. It would also be useful to examine whether women who screen positive are referred, whether they receive care, and whether services are available in their communities. The Virginia Department of Health, healthcare systems, community organizations, and maternal-health advocates could work together to improve screening, referral, and follow-up.

Support should begin before pregnancy and continue after delivery. Stress, limited social support, unmet material needs, and cultural experiences can all affect perinatal mental health.[13,14] This means that the response cannot stop at screening.[15] Programs should use validated tools, document referrals, follow up with patients, and examine differences in access and treatment. Rural communities may also need tele-mental-health services, transportation support, more trained providers, and partnerships with organizations that women already know and trust.

Limitations

This analysis cannot explain why the estimates changed. PRAMS information is self-reported, and state-level estimates may hide important differences between groups. The three indicators also measure different periods and should not be treated as the same clinical diagnosis.

Conclusion

Depression before pregnancy, depression during pregnancy, and postpartum depressive symptoms all increased among Virginia women. The largest increase occurred during pregnancy. These findings support screening at more than one point, along with reliable referrals, accessible treatment, and continued follow-up. Addressing perinatal depression across the full period before, during, and after pregnancy may improve outcomes for women, infants, and families.

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